UTI AND PYELONEPHRITIS IN PREGNANCY: WHAT TO DO NOW

FD-ANT-UTI · Companion to UTI and pyelonephritis in pregnancy · ACOG CC No. 4 · IDSA 2019 · ACOG CO 797 / 717 · SMFM #67 · Labor and delivery / OBED · Decision support only, not unit policy

CHRISTUS St. Michael / OBHG · Texarkana

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HOW TO READ THIS CHART: start by choosing In labor / admitted or OB ED triage. **Doses, regimens and durations live on the companion card; this chart lists none.** Every treatment node says see ANT-UTI card. Pregnant pyelo is always admitted.

SHADING KEY: WHITE = home / ASB or cystitis · LIGHT GRAY = first step / screen · MID GRAY = admit / inpatient pyelo · DARK GRAY = sepsis — SYS-03 first

START: IN LABOR / ADMITTED, OR OB ED TRIAGE?

Choose the path that matches how she presents. Pregnant or postpartum: GYNB-06 is never used.

IN LABOR / ADMITTED · WHITE

Sepsis screen first (CSM tool). Positive → node 5. Fever in labor: IAI (LND-04) first; send UA + culture. Confirmed pyelo → node 4; see ANT-UTI card. GBS bacteriuria any count → IAP. Postpartum inpatient → node 6.

OB ED TRIAGE · WHITE

Not in labor (any GA) or postpartum ≤12 wk. Continue to the OB ED branch below.

1. UNIVERSAL FIRST STEP · LIGHT GRAY

Vitals with sepsis screen (CSM tool). GA/viability; FHT or EFM/toco. Contractions, bleeding, ROM or abnormal FHR → L&D; in parallel. Sepsis screen positive or any red flag → node 5 (SYS-03) now. Otherwise: 2 ASB, 3 cystitis, or 4 pyelo.

2. ASB · WHITE

Uropathogen ≥100,000 CFU/mL: treat, culture-directed — see ANT-UTI card. Vaginal flora only: no treatment. GBS at ANY count: chart flag for IAP; antepartum treatment only if ≥100,000 or symptomatic — see ANT-UTI card. Symptoms → node 3 or 4. Home → node 7.

3. ACUTE CYSTITIS · WHITE

UA + culture before the first dose. Pyuria with negative culture, or vaginal symptoms → NAAT and ANT-STI (first alternate). Treat empirically — see ANT-UTI card. Fever, CVA tenderness or vomiting → node 4. Home if afebrile and tolerating PO → node 7.

4. PYELONEPHRITIS — ALWAYS ADMIT IN PREGNANCY · MID GRAY

Fever ≥38.0 °C + urine findings + flank pain/CVA tenderness (± N/V). No outpatient branch in pregnancy. UA + culture before the first IV dose (do not delay the dose). Start IV antibiotics in OBED — see ANT-UTI card. Measured fluids — ARDS/pulmonary edema risk. Clinically improved → oral step-down and total course — see ANT-UTI card. Not improving at 48–72 h → recheck culture; renal ultrasound; urology / ID as indicated. Septic → node 5.

5. SEPSIS RED FLAGS — SYS-03 FIRST · DARK GRAY

Open SYS-03 maternal sepsis flow first; ANT-UTI is the source card (antibiotic choice, source control). Cultures + lactate, antibiotics ideally within 1 h, crystalloid and pressor per SYS-03. Obstruction/stone/abscess → urology now. Sepsis alone is not an indication for delivery.

6. POSTPARTUM (≤12 WK) · LIGHT GRAY

No ASB screening or treatment postpartum. Cystitis → node 3 (lactation check on the card). Admit pyelo if any admit criterion (sepsis/red flag, unable to take PO, vomiting, immunocompromise, obstruction/stone, failed oral therapy, unreliable follow-up, within the delivery admission or ≤1 wk after cesarean). **House practice:** well-appearing, no admit criteria — IV dose plus oral therapy with follow-up in 24–48 h; see ANT-UTI card. Pregnant pyelo is still always admitted. GYNB-06 is still not used.

7. DISPOSITION AND RETURN PRECAUTIONS · WHITE

Home (ASB, cystitis, or postpartum per node 6): afebrile, no CVA tenderness, tolerating PO, call-back named, GBS flag if applicable. Admit: any pyelo in pregnancy; sepsis; postpartum admit criterion. Return for fever, flank pain, vomiting, contractions, bleeding, leaking, decreased FM, shortness of breath, or symptoms not better in 48 h. **House practice:** routine test-of-cure culture 2 weeks after completing treatment for ASB or cystitis (ACOG and IDSA do not require it); after pyelo, test-of-cure culture 2 weeks after completing treatment. **House practice:** routine suppression after a single pyelo, until 4–6 weeks postpartum — see ANT-UTI card.